

## REFERRAL / CONSULTATION REQUEST FORM

Shenandoah Valley Regional Medical Associates  
2201 Valley Health Drive, Suite 110 | Woodstock, VA 22664  
Tel: (540) 459-8800 Fax: (540) 459-8822  
NPI: 1427836509

DATE OF REQUEST: 10/03/2024 PRIORITY: [ X ] Routine [ ] Urgent

REFERRING PROVIDER: James R. Kowalski, MD  
(Internal Medicine / Primary Care) FAX TO: Johns Hopkins Neurology  
Attn: Referral Intake

### PATIENT INFORMATION:

Name: Delaney, Margaret A. DOB: 04/17/1968 (Age: 56)  
MRN (local): 0048821-A Sex: Female  
Insurance: Aetna PPO #AET-8821-0047 Auth #: PENDING

### REASON FOR REFERRAL / CLINICAL QUESTION:

Pt referred for evaluation of recurrent episodes of loss of consciousness, approximately 4-5 episodes over past 8 months. Etiology unclear at this time. Cardiology w/u largely unremarkable (see attached). ? Vasovagal vs. cardiac arrhythmia vs. other. Pt continues to have episodes despite conservative mgmt. Requesting specialist eval and further workup as appropriate.

\*\* See attached clinical notes, labs, imaging \*\*

### RELEVANT DIAGNOSES (per referring provider):

- Syncope, unspecified (R55)
- Hypertension (I10)
- Hyperlipidemia (E78.5)
- GERD (K21.0)
- Anxiety disorder, NOS (F41.1)

Referring Provider Signature: Date: 10/03/2024

/s/ James R. Kowalski, MD [Electronically signed via EHR]

### FOR OFFICE USE ONLY:

Received: \_\_\_\_\_ Triaged by: \_\_\_\_\_ Appt Date: \_\_\_\_\_  
Assigned to: \_\_\_\_\_ Notes: \_\_\_\_\_

**PATIENT DEMOGRAPHICS & INSURANCE INFORMATION**

Shenandoah Valley Regional Medical Associates | Printed: 10/03/2024 14:22

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**DEMOGRAPHIC INFORMATION**

Last Name: Delaney                      First: Margaret                      MI: A  
DOB: 04/17/1968                      Age: 56                      Sex: F  
SSN: \*\*\*-\*\*-7743                      Race: White                      Ethnicity: Non-Hispanic

Address:  
814 Orchard Hill Road                      City: Edinburg  
State: VA                      Zip: 22824

Home Phone: (540) 984-2271                      Cell: (540) 984-0093  
Email: m.delaney1968@gmail.com                      Preferred Contact: Cell

Emergency Contact: Patrick Delaney                      Rel: Spouse  
(husband)  
Phone: (540) 984-0088

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**PRIMARY INSURANCE**

Carrier: Aetna                      Plan Type: PPO  
Member ID: AET-8821-0047                      Group: 7741200  
Policy Holder: Margaret A. Delaney                      Relationship: Self  
Employer: Shenandoah County Schools                      Effective Date: 01/01/2022  
(retired)

**SECONDARY INSURANCE**

None on file

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**PHARMACY INFORMATION**

Preferred Pharmacy: Rite Aid #4471  
Address: 104 S Main St, Woodstock VA Phone: (540) 459-2200  
22664

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**PCP / CARE TEAM**

PCP: James R. Kowalski MD                      NPI: 1427836509  
Cardiology: Elena V. Marchetti MD                      NPI: 1609374821  
[Shenandoah Valley Cardiology, Winchester VA]

\*\* Patient authorizes release of records. Form on file dated 09/28/2024 \*\*

**SHENANDOAH VALLEY REGIONAL MEDICAL ASSOCIATES**

James R. Kowalski MD | Internal Medicine  
2201 Valley Health Drive, Ste 110, Woodstock VA 22664  
Tel: 540-459-8800

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**OFFICE VISIT NOTE**

|                                |                                 |
|--------------------------------|---------------------------------|
| Patient: Delaney, Margaret A.  | MRN: 0048821-A                  |
| DOB: 04/17/1968                | Date of Service: 10/01/2024     |
| Provider: James R. Kowalski MD | Visit Type: Established Patient |
| Signed: 10/02/2024 09:14       | Attestation: Complete           |

**CHIEF COMPLAINT:**

"I blacked out again last week. This is the fourth or fifth time now."

**HPI:**

Ms. Delaney is a 56 year old female with a history of hypertension, hyperlipidemia, GERD, and anxiety who presents today for follow-up regarding recurrent syncopal episodes. She first presented to this office approximately 8 months ago (see prior note 02/14/2024) after her first episode. At that time a cardiology referral was placed.

Today she reports that she had another episode on 09/25/2024. She describes it as occurring while she was in the kitchen in the morning, approximately 1 hour after waking. She states she had "a funny smell" right before things went dark - she describes it as burning rubber or something electrical. She denies any preceding lightheadedness or dizziness. No palpitations noted prior to event per patient. Husband witnessed the episode and reports she did not fall but rather "slumped" against the counter and then slid to the floor. He states her eyes were "open but rolled back" and her right arm was shaking for maybe 20-30 seconds. She was confused for several minutes afterward - he says she was asking repeated questions. She denies any tongue biting or urinary incontinence but husband says he is not sure about the latter. Duration of LOC approximately 1-2 minutes per husband.

She reports a similar preceding smell with at least 2 of the prior episodes. Episodes are not consistently associated with standing or positional changes - most have occurred at rest or with minimal activity. No consistent triggers identified. She has not driven since her second episode.

No recent illness. No changes to medications. No alcohol or illicit substance use. No significant head trauma. No family history of seizures per patient.

**REVIEW OF SYSTEMS:**

Positive: olfactory aura preceding events, postictal confusion (per husband), limb movement during episode (per husband)  
Negative: headache, vision changes, fever, chest pain, dyspnea, palpitations, incontinence (patient uncertain)

PAST MEDICAL HISTORY:

1. Hypertension - on lisinopril
2. Hyperlipidemia - on atorvastatin
3. GERD - on omeprazole
4. Anxiety disorder NOS - on sertraline
5. Osteoarthritis, bilateral knees - occasional ibuprofen use
6. Right knee meniscal repair 2019
7. Appendectomy 1988
8. s/p cholecystectomy 2011
9. ? Iron deficiency anemia (mild, 2022 - not formally treated)

**MEDICATIONS:**

See attached medication reconciliation sheet

**ALLERGIES: NKDA**

**SOCIAL HISTORY:**

Former middle school teacher, retired 2021. Non-smoker. Occasional alcohol (1-2 glasses wine/week). No illicit drugs. Married. Two adult children. Lives in Edinburg, VA.

PHYSICAL EXAM:

VS: BP 138/84 mmHg HR 72 bpm RR 16 SpO2 98% RA  
Temp: 98.4 F Wt: 162 lbs Ht: 5'4"

Gen: Alert and oriented x3, in no acute distress

HEENT: Normocephalic, atraumatic. PERRL. EOMI.

Neck: Supple, no JVD, no carotid bruits

Cardiovascular: RRR, no m/r/g, 2+ peripheral pulses

Resp: CTAB, no wheeze/rales/rhonchi

Abdomen: Soft, NT/ND. Well-healed RLQ and RUQ scars.

Neuro: CN II-XII grossly intact. Motor 5/5 bilateral UE/LE.

Sensation grossly intact. No tremor at rest. Gait normal.

No focal deficits noted on exam today.

MSK: No acute joint swelling.

Skin: No rash or lesions.

**ASSESSMENT AND PLAN:**

1. Recurrent loss of consciousness, etiology TBD
  - Cardiology w/u (Marchetti) largely unremarkable per last correspondence (Echo WNL, Holter x 30 days - no significant arrhythmia captured, tilt table negative x2)
  - Husband's description of event concerning for possible seizure activity (eye deviation, unilateral arm shaking, postictal confusion, olfactory aura)
  - Will refer to neurology for further evaluation
  - Pt instructed not to drive pending specialist eval
  - Order routine EEG through Shenandoah Valley Radiology - pending
2. HTN: Stable on lisinopril. Continue.
3. Hyperlipidemia: Due for fasting lipid panel - ordered

4. GERD: Stable on omeprazole. Continue.
5. Anxiety: Patient notes anxiety is "about the same." Continue sertraline.

RTC 3 months or sooner as needed. Neurology referral submitted.

Electronically signed: James R. Kowalski MD 10/02/2024 09:14

**\*\* COPY OF RECORD - SHENANDOAH VALLEY CARDIOLOGY \*\***

Elena V. Marchetti MD | Cardiology  
88 Medical Center Blvd, Suite 220, Winchester VA 22601  
Tel: 540-722-4400 Fax: 540-722-4422

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**CARDIOLOGY FOLLOW-UP NOTE**

Patient: DELANEY, MARGARET A. DOB: 04/17/1968  
MRN (Cardiology): CRD-20240314 Encounter Date: 07/22/2024  
Referring Provider: Kowalski, James R. MD

**REASON FOR VISIT:**

Follow up after holter monitor and tilt table testing. Pt referred by PCP for recurrent syncopal episodes x 3 at time of initial cardiology evaluation (see note 04/08/2024).

**INTERVAL HISTORY:**

Pt reports one additional episode since last visit (approximately 6 weeks ago). Similar description - occurred while seated reading. Preceding smell again reported. Approx 1 min of LOC. Husband reports some "shaking" of her right arm during event. Postictal confusion for 5-10 minutes. No preceding palpitations, no chest pain, no diaphoresis prior to event.

**RESULTS REVIEWED:**

- 30-day event monitor (Zio Patch, 05/14/2024 - 06/13/2024):  
No significant arrhythmia captured during monitoring period.  
Two patient-triggered events - review of corresponding rhythm showed normal sinus rhythm at times of symptom reporting.  
No evidence of SVT, VT, AV block, or significant pauses.
- Transthoracic Echo (04/10/2024):  
EF 60-65%. No WMA. No significant valvular disease.  
No intracardiac thrombus. Trace TR.
- Head-up Tilt Table Test x2 (04/30 and 05/15/2024):  
Both studies negative. No vasovagal response elicited at 70-degree tilt up to 45 minutes. Isoproterenol not utilized.

**IMPRESSION:**

Recurrent loss of consciousness with negative cardiac workup to date. Echocardiogram, extended cardiac monitoring, and tilt table testing all unremarkable. Structural and arrhythmic cardiac etiologies appear unlikely based on current data.

I share the PCP's concern that the clinical description (olfactory prodrome, witnessed unilateral limb jerking, prolonged postictal confusion) is atypical for a primary cardiac etiology. These features may be more consistent with a neurologic process. Would strongly recommend neurology evaluation. I do not feel additional cardiac workup is indicated at this time.

Will discharge from cardiology care pending neurology assessment.

Please feel free to re-refer if new cardiac symptoms emerge.

Electronically signed: Elena V. Marchetti MD 07/23/2024 11:40

-- [COPY FORWARDED FROM CARDIOLOGY EHR - PLEASE DIRECT INQUIRIES --  
-- TO ORIGINATING PROVIDER] --

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**\*\* FACSIMILE TRANSMISSION \*\***  
**SHENANDOAH MEMORIAL HOSPITAL | EMERGENCY DEPARTMENT**  
759 S. Main St, Woodstock VA 22664  
Tel: 540-459-1100 Fax: 540-459-1199  
*CONFIDENTIAL - FOR AUTHORIZED RECIPIENT ONLY*

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**EMERGENCY DEPARTMENT VISIT SUMMARY**

Patient Name: DELANEY MARGARET ANN  
DOB: 04-17-1968 MRN: SMH-331094  
Arrival: 09/25/2024 09:47 Disposition: 09/25/2024 15:22  
Mode of Arrival: Private vehicle  
Chief Complaint: "Passed out"  
Attending: Kevin P. Oduya MD | EM Physician

**TRIAGE NOTE (09:52):**

56F brought in by husband after witnessed loss of consciousness at home. Pt is alert and oriented on arrival. C/o mild headache and fatigue. Husband reports she "blacked out" for about a minute, right arm was shaking. Prior hx of similar episodes x 3-4.

VS on arrival: BP 152/90, HR 88, RR 18, SpO2 99%, Temp 98.6

**ED PHYSICIAN NOTE:**

HPI: 56 year old female with PMH as above presenting after witnessed syncopal episode this morning. Per husband, pt was standing in kitchen when she became unresponsive. He reports her eyes were open and deviated upward. Right arm shaking noted x approximately 30 sec. She did not fall - she slumped against counter. No tonic-clonic activity of whole body per husband. Confused for several minutes after. Pt reports smelling something "burnt" immediately before episode. This is her 4th or 5th episode - prior workup per her PCP includes: echo (WNL), holter (no arrhythmia), tilt table (x2, negative). Pt denies chest pain, SOB, palpitations. No incontinence per pt.

PMH: HTN, HLD, GERD, anxiety, OA knees, s/p appy, s/p chole  
Medications: lisinopril 10mg, atorvastatin 20mg, omeprazole 20mg,  
sertraline 100mg, ibuprofen PRN  
Allergies: NKDA

**PHYSICAL EXAM (10:14):**

Gen: Alert, oriented x3. Mildly fatigued appearing.  
HEENT: No head trauma. PERRL 3->2 bilat. EOMI. No nystagmus.  
Neuro: CN intact. Motor 5/5 all extremities. Sensation intact.  
DTRs 2+ symmetric. No Babinski. Mild confusion resolved by time of full exam. Gait slightly unsteady on initial assessment, improved.  
CV: RRR. No murmur.

**DIAGNOSTIC RESULTS:**

EKG (10:02): NSR, rate 82. Normal axis. Normal intervals. No ST



changes. No delta waves. QTc 430ms.  
BMP: Na 139 / K 4.1 / Cl 102 / Bicarb 24 / BUN 14 / Cr 0.78 /  
Glucose 94  
CBC: WBC 7.4 / Hgb 12.1 / Hct 36.4 / Plt 218  
[Hgb slightly low - noted, possibly chronic]  
Troponin: <0.01 (negative)  
Mg: 2.0 | Phos: 3.2 | Ca: 9.2  
LFTs: Within normal limits  
TSH: 2.14 (normal)  
Urine HCG: Negative  
Urine drug screen: Negative  
Serum EtOH: Not detected

CT Head without contrast (11:30):

No acute intracranial abnormality. No hemorrhage. No mass effect.  
No midline shift. [See separate radiology report - attached]

**ASSESSMENT AND DISPOSITION:**

56F with recurrent LOC x 4-5 episodes over ~8 months, prior cardiac workup negative. Today's episode with olfactory aura, unilateral arm shaking, postictal confusion. Presentation does not fit cleanly into vasovagal or cardiogenic syncope. CT head negative for acute pathology. Neurology not available for inpatient consult at this facility.

Admitting neurology or obtaining outpatient EEG / neurology consult recommended. Discussed with patient and husband at length. Patient declines inpatient admission. Will arrange urgent outpatient follow-up with PCP. Instructed to not drive.

Diagnosis at discharge: Syncope and collapse, unspecified (R55)

[Note: diagnosis entered per ED coding requirements; clinical picture may represent seizure - recommend outpt neurology eval]

Discharge instructions reviewed with pt and husband.

Kevin P. Oduya MD | Signed 09/25/2024 15:08

**SHENANDOAH VALLEY REGIONAL MEDICAL ASSOCIATES**

James R. Kowalski MD | Internal Medicine  
2201 Valley Health Drive, Ste 110, Woodstock VA 22664

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**OFFICE VISIT NOTE**

Patient: Delaney, Margaret A. MRN: 0048821-A  
DOB: 04/17/1968 Date of Service: 02/14/2024  
Provider: James R. Kowalski MD Visit Type: Established Patient  
Signed: 02/15/2024 08:03

**CHIEF COMPLAINT:** "I fainted last month."

**HPI:**

Ms. Delaney is a 56F with h/o HTN, HLD, GERD, and anxiety who presents today to establish care after moving to Shenandoah County and also to discuss a syncopal episode she experienced in January.

She reports she was at home, was walking from the bedroom to the hallway, when she "just went out." Husband witnessed - reports she fell to the ground. He is not sure how long she was unconscious. She remembers waking up on the floor. She is not certain if there was any prodrome. She denies any palpitations beforehand. No incontinence. No prior episodes. No recent illness. No new meds. No family history of sudden death or arrhythmia. Denies any focal weakness, vision changes, or confusion after the event.

[Note: Patient report at this visit is less detailed than later visits. Husband not present.]

**PAST MEDICAL HISTORY:**

- HTN (dx ~10 years ago, on lisinopril)
- Hyperlipidemia (on atorvastatin)
- GERD (on omeprazole)
- Anxiety NOS (on sertraline)
- OA bilateral knees
- Meniscal repair right knee 2019
- Appendectomy remote
- Cholecystectomy 2011

Prior PCP: Dr. Annette Voss, Harrisonburg VA (records requested, not yet received as of this note)

**PHYSICAL EXAM:**

VS: BP 144/86, HR 74, Wt 163 lbs  
Gen: NAD. Alert and oriented.  
CV: RRR. No m/r/g. No carotid bruits.  
Neuro: Grossly intact. No focal deficits.  
Ortho: Mild crepitus bilateral knees. No effusion.

**LABS (ordered today):**

CBC, CMP, Lipid panel, TSH, B12, folate, iron studies - pending

EKG ordered - results below

EKG (in office): NSR. Rate 74. Normal axis. PR 162ms. QRS 88ms. QTc 428ms. No ST changes. No pre-excitation pattern.

**ASSESSMENT:**

1. Syncope - single episode, etiology unclear. Rule out cardiac cause. Refer to cardiology for evaluation.
2. HTN - mildly elevated today. Increase lisinopril from 5mg to 10mg daily. Recheck BP in 4-6 weeks.
3. Hyperlipidemia - labs pending
4. GERD - continue omeprazole
5. Anxiety - continue sertraline 100mg
6. OA knees - recommend tylenol or topical diclofenac to minimize NSAID use given BP concern

Cardiology referral placed (Marchetti). RTC 6-8 weeks.

Electronically signed: James R. Kowalski MD 02/15/2024 08:03

----- AUTO-FORWARDED COPY - SHENANDOAH VALLEY RMA EHR -----

**SHENANDOAH VALLEY REGIONAL LAB SERVICES**  
Affiliated with Shenandoah Memorial Hospital  
759 S Main St, Woodstock VA 22664 | CLIA: 49D2094771  
Fax: 540-459-1155

**LABORATORY REPORT**

Patient: DELANEY, MARGARET A.      DOB: 04/17/1968  
MRN: 0048821-A      Ordering MD: Kowalski, J.  
Collected: 02/14/2024 10:41      Reported: 02/15/2024 07:22

| TEST                                 | RESULT     | REFERENCE  | FLAG |
|--------------------------------------|------------|------------|------|
| <b>COMPREHENSIVE METABOLIC PANEL</b> |            |            |      |
| Sodium                               | 139 mEq/L  | 136-145    |      |
| Potassium                            | 4.0 mEq/L  | 3.5-5.1    |      |
| Chloride                             | 101 mEq/L  | 98-107     |      |
| CO2                                  | 25 mEq/L   | 22-29      |      |
| BUN                                  | 16 mg/dL   | 7-25       |      |
| Creatinine                           | 0.82 mg/dL | 0.5-1.1    |      |
| eGFR                                 | >60 mL/min | ≥60        |      |
| Glucose                              | 91 mg/dL   | 70-99      |      |
| Calcium                              | 9.0 mg/dL  | 8.5-10.5   |      |
| Total Protein                        | 7.2 g/dL   | 6.3-8.2    |      |
| Albumin                              | 4.1 g/dL   | 3.5-5.0    |      |
| Total Bilirubin                      | 0.6 mg/dL  | 0.2-1.2    |      |
| ALT                                  | 22 U/L     | 7-35       |      |
| AST                                  | 24 U/L     | 13-35      |      |
| Alk Phos                             | 78 U/L     | 44-147     |      |
| <b>COMPLETE BLOOD COUNT</b>          |            |            |      |
| WBC                                  | 7.1 K/uL   | 4.5-11.0   |      |
| RBC                                  | 4.01 M/uL  | 3.80-5.20  |      |
| Hemoglobin                           | 12.0 g/dL  | 11.5-15.5  | L    |
| Hematocrit                           | 36.0 %     | 34.0-45.0  |      |
| MCV                                  | 78.2 fL    | 80.0-100.0 | L    |
| MCH                                  | 25.6 pg    | 27.0-33.0  | L    |
| MCHC                                 | 32.8 g/dL  | 31.5-35.7  |      |
| RDW                                  | 14.8 %     | 11.5-14.5  | H    |
| Platelets                            | 224 K/uL   | 150-400    |      |
| Neutrophils                          | 4.4 K/uL   | 1.8-7.5    |      |
| Lymphocytes                          | 1.9 K/uL   | 1.0-4.8    |      |
| <b>IRON STUDIES</b>                  |            |            |      |
| Serum Iron                           | 44 ug/dL   | 50-170     | L    |
| TIBC                                 | 388 ug/dL  | 250-370    | H    |

|                              |            |           |   |
|------------------------------|------------|-----------|---|
| Ferritin                     | 8 ng/mL    | 12-150    | L |
| <b>THYROID</b>               |            |           |   |
| TSH                          | 2.14 mIU/L | 0.40-4.50 |   |
| <b>LIPID PANEL (fasting)</b> |            |           |   |
| Total Cholesterol            | 214 mg/dL  | <200      | H |
| LDL                          | 138 mg/dL  | <100      | H |
| HDL                          | 48 mg/dL   | >50       | L |
| Triglycerides                | 142 mg/dL  | <150      |   |
| <b>VITAMINS</b>              |            |           |   |
| Vitamin B12                  | 344 pg/mL  | 200-900   |   |
| Folate                       | 10.2 ng/mL | 2.7-17.0  |   |

*L = Low | H = High | Results verified by lab director*

Flagged values: Hemoglobin slightly low; Iron studies consistent with iron deficiency. MCV/MCH suggest microcytic pattern.  
LDL elevated - discussed at visit.

**SHENANDOAH VALLEY RADIOLOGY ASSOCIATES**

Teleradiology Services - Woodstock / Winchester / Front Royal  
CLIA: 49R0041122 | Fax: 540-459-8880

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**RADIOLOGY REPORT #1**

|                                |                                  |
|--------------------------------|----------------------------------|
| Patient: DELANEY, MARGARET A.  | DOB: 04/17/1968                  |
| MRN: SMH-331094                | Ordering: Oduya, K. MD (ED)      |
| Exam: CT HEAD W/O CONTRAST     | Accession: SMH-2024-09-331094    |
| Date of Exam: 09/25/2024 11:30 | Date of Report: 09/25/2024 12:14 |

**INDICATION:**

56F with loss of consciousness, shaking of right arm, postictal confusion. Evaluate for intracranial pathology.

**TECHNIQUE:**

Non-contrast CT of the brain. 5mm axial sections with reformats.

**FINDINGS:**

Brain parenchyma: No acute infarct. No hemorrhage. No mass or mass effect. No midline shift. Gray-white differentiation preserved. No cerebral edema.

Ventricles and cisterns: Ventricles normal in size and configuration. Basal cisterns patent.

Posterior fossa: Cerebellum and brainstem unremarkable.

Calvarium/Soft Tissues: No calvarial fracture. Mild paranasal sinus mucosal thickening in the left maxillary sinus - likely inflammatory. Aerosolized secretions noted.

**IMPRESSION:**

1. No acute intracranial abnormality.
2. Mild left maxillary sinus disease - likely inflammatory/infectious; no separate intervention needed unless clinically indicated.

LIMITATIONS: CT is limited in detection of cortical lesions, early infarct, and subtle mesial temporal pathology. If clinical suspicion remains, MRI brain with and without contrast may be considered.

Radiologist: Priya Subramaniam MD | Signed 09/25/2024 12:14

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**RADIOLOGY REPORT #2**

|                                  |                             |
|----------------------------------|-----------------------------|
| Patient: DELANEY, MARGARET A.    | DOB: 04/17/1968             |
| MRN: 0048821-A                   | Ordering: Kowalski, J. MD   |
| Exam: RIGHT KNEE X-RAY (3 views) | Accession: SVR-2024-04-0481 |
| Date of Exam: 04/14/2024         | Date of Report: 04/14/2024  |

**INDICATION: Right knee pain, history of meniscal repair.**

**FINDINGS:**

Mild-moderate medial compartment joint space narrowing. Osteophyte formation medial femoral condyle and medial tibial plateau. Soft tissues unremarkable. No acute fracture. Prior surgical changes noted. Patellofemoral joint preserved.

**IMPRESSION: Mild-moderate medial compartmental osteoarthritis.**

No acute fracture or joint effusion.

Radiologist: G. Hoffmann MD | Signed 04/14/2024 16:02

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**RADIOLOGY REPORT #3**

Patient: DELANEY, MARGARET A.

DOB: 04/17/1968

MRN: CRD-20240314

Ordering: Marchetti, E. MD

Exam: TRANSTHORACIC ECHOCARDIOGRAM

Date: 04/10/2024

**INDICATION: Syncope evaluation. Assess for structural cardiac disease.****FINDINGS:**

LV: Normal size and systolic function. EF 60-65%. No regional wall motion abnormalities.

RV: Normal size and function. TAPSE 2.1 cm.

Valves: Mitral leaflets thin, no significant MR. Aortic valve trileaflet, no stenosis, trace AR. Tricuspid valve with trace TR.

Pulmonic valve: Normal.

Pericardium: No effusion.

Aortic root: 3.2 cm (normal). Ascending aorta visualized, normal.

IVC: Normal, collapsible >50%.

**IMPRESSION:**

1. Normal left ventricular systolic function, EF 60-65%.
2. No significant valvular disease.
3. No structural cardiac explanation for syncope identified.

Signed: W. Okafor MD (Reading Cardiologist) 04/10/2024 17:30

## MEDICATION RECONCILIATION

Shenandoah Valley Regional Medical Associates

Printed: 10/03/2024 14:22 | Reconciled at: 10/01/2024 visit

Patient: DELANEY, MARGARET A. DOB: 04/17/1968 MRN: 0048821-A

| MEDICATION               | DOSE    | FREQ  | ROUTE   | INDICATION / NOTES                                                |
|--------------------------|---------|-------|---------|-------------------------------------------------------------------|
| Lisinopril               | 10 mg   | Daily | PO      | Hypertension. Increased from 5mg 02/2024.                         |
| Atorvastatin             | 20 mg   | QHS   | PO      | Hyperlipidemia. LDL 138 - may need uptitration.                   |
| Omeprazole               | 20 mg   | Daily | PO      | GERD. Long-term use - consider H. pylori testing per GI.          |
| Sertraline               | 100 mg  | Daily | PO      | Anxiety NOS. At this dose x ~4 years.                             |
| Ibuprofen                | 400 mg  | PRN   | PO      | OA pain. Use limited d/t HTN. Advised topical alternative.        |
| Diclofenac sodium gel 1% | Apply   | PRN   | Topical | Right knee OA. Recommended as NSAID alternative 02/2024.          |
| Ferrous sulfate          | 325 mg  | BID   | PO      | Iron deficiency - STARTED 03/2024. Tolerating per pt.             |
| Calcium carbonate + D3   | 600/400 | Daily | PO      | OTC supplement - patient self-initiated. Noted on reconciliation. |
| Multivitamin (OTC)       | 1 tab   | Daily | PO      | Self-initiated. No concerns.                                      |
| Melatonin 5mg (OTC)      | 5 mg    | QHS   | PO      | Self-initiated for sleep difficulty. Not prescribed.              |
| Aspirin 81mg (OTC)       | 81 mg   | Daily | PO      | Self-initiated. No clear indication - discussed at visit.         |

### ALLERGIES:

NKDA - No known drug allergies (confirmed with patient 10/01/2024)

### DISCONTINUED / HISTORICAL MEDICATIONS:

- Lisinopril 5mg -- dose increased to 10mg 02/14/2024
- Amlodipine 5mg -- previously prescribed by Dr. Voss (prior PCP, Harrisonburg) -- DC'd ~2022 per patient; records not available
- Alprazolam 0.25mg PRN -- per patient was prescribed briefly in 2020 for procedure-related anxiety; no refills; not current

\*\* Reconciliation confirmed with patient. Discrepancies may exist with prior provider records not yet received. \*\*

Reconciled by: S. Hartman RN | 10/01/2024 08:44



**SHENANDOAH VALLEY NEUROLOGY DIAGNOSTICS**  
Electrodiagnostic Services - Reading Neurologist  
Fax: 540-459-8855 | Tel: 540-459-8850

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**ROUTINE EEG REPORT**

|                               |                               |
|-------------------------------|-------------------------------|
| Patient: DELANEY, MARGARET A. | DOB: 04/17/1968               |
| MRN: 0048821-A                | Order: Kowalski, J. MD        |
| Test Date: 10/02/2024 09:00   | Report Date: 10/02/2024 16:44 |
| Tech: Nguyen, H. (R.EEG T.)   | Duration: 22 minutes          |

**INDICATION:**

Recurrent loss of consciousness with olfactory prodrome, witnessed limb jerking, and postictal confusion. R/O epileptiform activity.

**TECHNIQUE:**

Standard 10-20 electrode placement. Recording during wakefulness and drowsiness. Hyperventilation performed x 3 minutes. Photoc stimulation performed at 1, 2, 4, 6, 8, 10, 12, 14, 16, 18, 20 Hz. No sleep captured during this recording.

**FINDINGS:**

Background: Posterior dominant rhythm at 9-10 Hz, well-formed and reactive. Anteroposterior gradient preserved. No focal slowing appreciated on this recording.

Hyperventilation: No significant change in background activity. No paroxysmal activity elicited.

Photoc stimulation: No photoparoxysmal response.

Epileptiform features: No definite epileptiform discharges captured on this recording.

Artifacts: Muscle artifact and eye movement artifact noted intermittently, limiting interpretation during those segments.

**IMPRESSION:**

1. No epileptiform discharges captured on this routine interictal EEG.

NOTE: A normal routine EEG does not exclude a diagnosis of epilepsy. The sensitivity of a single routine EEG for detecting epileptiform activity in patients with epilepsy is approximately 40-50%. Extended monitoring, sleep-deprived, or ambulatory EEG may increase yield. Clinical context and specialist evaluation are essential for diagnostic determination.

Reading Neurologist: R. Jain MD (Telehealth/Teleneuro),  
Mid-Atlantic Neurology Reading Services  
Signed: 10/02/2024 16:44

\*\* This report was generated by an outside reading service and may not reflect full clinical consultation. \*\*

**SHENANDOAH MEMORIAL HOSPITAL**  
**DISCHARGE SUMMARY - EMERGENCY DEPARTMENT**  
759 S. Main St, Woodstock VA 22664 | Fax: 540-459-1199

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|                                |                             |
|--------------------------------|-----------------------------|
| Patient: DELANEY, MARGARET ANN | MRN: SMH-331094             |
| DOB: 04/17/1968                | Sex: F                      |
| Arrival: 09/25/2024 09:47      | Discharge: 09/25/2024 15:22 |
| Attending: Kevin P. Oduya MD   | Service: Emergency Medicine |

**PRINCIPAL DIAGNOSIS AT DISCHARGE:**

R55 - Syncope and collapse, unspecified  
[Provider note: see ED physician note for clinical commentary]

**SECONDARY DIAGNOSES:**

I10 - Essential hypertension  
E78.5 - Hyperlipidemia  
D50.9 - Iron deficiency anemia, unspecified

**PROCEDURES PERFORMED:**

- 12-lead ECG
- CT head without contrast
- IV access (peripheral, right AC)
- Labs: BMP, CBC, troponin, TSH, Mg, Phos, Ca, LFTs, UDS, EtOH

**HOSPITAL COURSE:**

56F with history of hypertension, hyperlipidemia, GERD, and anxiety presenting after witnessed loss of consciousness at home with preceding olfactory aura, right arm shaking per husband, and postictal confusion. This is the 4th or 5th episode over approximately 8 months. Prior outpatient cardiac workup (echo, 30-day monitor, tilt table x2) was negative per referring PCP.

Patient evaluated in ED. Vitals significant for mildly elevated BP on arrival (152/90), which normalized with rest. EKG showed normal sinus rhythm without arrhythmia or pre-excitation. Lab work notable for mild anemia (Hgb 12.1) and low hemoglobin consistent with prior iron deficiency - not felt to be acutely contributing to episode. Troponin negative. Glucose normal. Electrolytes within normal limits. Drug screen negative.

CT head without contrast: No acute intracranial abnormality. (See radiology report for details.)

Patient observed in ED x approximately 5 hours. Neurologically intact at time of discharge. Confusion resolved. Patient ambulatory without difficulty at discharge. Hemodynamically stable.

Neurology not available for acute consultation at this facility. Patient declined inpatient admission for further monitoring.

**DISCHARGE CONDITION: Good**

**DISCHARGE MEDICATIONS:**

No new prescriptions. Continue home medications as prescribed.  
(See medication list in records.)

**DISCHARGE INSTRUCTIONS:**

- Do NOT drive until cleared by a physician.
- Follow up with your primary care physician within 48-72 hours.
- Neurology referral strongly recommended - please discuss with PCP.
- Return to ED if: repeat loss of consciousness, chest pain, shortness of breath, focal weakness, vision loss, or any new concerning symptoms.

**FOLLOW-UP:**

- PCP (Kowalski): within 48-72 hours
- Neurology: TBD via PCP referral

Electronically signed: Kevin P. Oduya MD 09/25/2024 15:08

Attending Attestation: Complete

\*\* Document auto-generated by SMH EHR (MedHost v14.2) \*\*

\*\* Faxed to: Kowalski, J. MD on 09/26/2024 at 08:11 \*\*

**\*\* COPY OF RECORD - SHENANDOAH VALLEY CARDIOLOGY \*\***

Elena V. Marchetti MD | Cardiology

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**CARDIOLOGY NEW PATIENT NOTE**

Patient: DELANEY, MARGARET A.      DOB: 04/17/1968  
MRN (Cardiology): CRD-20240314      Date: 04/08/2024  
Referring: Kowalski, J. MD      Reason: Syncope evaluation

**HPI (COPIED FORWARD FROM REFERRAL - KOWALSKI 02/14/2024):**

[The following text was imported from the PCP referral.]  
56F, hx HTN, HLD, GERD, anxiety. Presents for evaluation of syncope.  
Episode in January - walking at home, witnessed by husband, LOC x  
uncertain duration. No prodrome reported at that visit. No incontinence.  
No palpitations prior. No family history of sudden death.

[Cardiology addendum:]

Since the initial PCP note, patient reports she has had two additional episodes. The second episode occurred in early March. The third episode occurred in late March. Both were witnessed by husband. She now reports that she noticed an unusual smell (described as like burning rubber) in the moments before both of the more recent episodes. This prodrome was not described at the initial PCP visit. Husband noted eye rolling and some arm movement during at least one episode. Duration of LOC approximately 1 minute. She describes feeling confused and tired for 15-20 minutes afterward.

No prior history of seizure or neurological diagnosis. No prior EEG or neuroimaging on record. No significant head trauma.

**CARDIAC ROS: Negative for exertional syncope, chest pain,**  
palpitations, dyspnea, orthopnea, PND, lower extremity edema.

**EXAM:**

BP 140/82 (right arm), 138/84 (left arm). HR 70. BMI 27.8.  
CV: RRR, S1/S2 normal. No murmurs. JVP 6cm. No edema.  
Neuro: Alert and oriented. No gross deficits. Normal gait.

**PLAN:**

Ordering: TTE, 30-day event monitor (Zio Patch), head-up tilt table.  
Will evaluate for structural and arrhythmic cardiac etiology.  
Clinical presentation is atypical for typical vasovagal syncope.  
Olfactory prodrome and postictal confusion noted - will complete cardiac workup but maintain low threshold for neurology referral.

[Per 07/2024 f/u: all studies negative - see note page 4]

Elena V. Marchetti MD | Signed 04/09/2024 13:20

----- END OF DOCUMENT -----



**MISCELLANEOUS RECORDS / ADDITIONAL LABS**

*[Auto-compiled by EHR export function - includes all labs on file]*

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**\*\* LIPID PANEL - REPEAT (ordered 10/01/2024 - results 10/02/2024) \*\***

Patient: DELANEY, MARGARET A. MRN: 0048821-A  
Collected: 10/02/2024 07:30 (fasting) Reported: 10/02/2024 10:14

|                    |           |             |                      |
|--------------------|-----------|-------------|----------------------|
| Total Cholesterol: | 188 mg/dL | (Ref: <200) | [WNL - improved]     |
| LDL Cholesterol:   | 112 mg/dL | (Ref: <100) | [H - still elevated] |
| HDL Cholesterol:   | 50 mg/dL  | (Ref: >50)  | [borderline]         |
| Triglycerides:     | 130 mg/dL | (Ref: <150) | [WNL]                |

Note: LDL improved from 138 mg/dL (02/2024) but remains above goal.  
May consider uptitrating statin or adding ezetimibe. Discussed at visit.

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**\*\* CBC - REPEAT (ordered 10/01/2024) \*\***

Collected: 10/02/2024 07:30

|               |                |               |
|---------------|----------------|---------------|
| WBC: 7.2 K/uL | Hgb: 12.3 g/dL | Hct: 37.0%    |
| MCV: 80.1 fL  | MCH: 26.8 pg   | Plt: 231 K/uL |

[Hemoglobin slightly improved from 12.0 in Feb 2024 - ferrous sulfate compliance confirmed. Continue.]

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**\*\* COLONOSCOPY NOTE (included in records export - likely not relevant) \*\***

From: Shenandoah Valley GI Associates  
Date: 08/14/2023  
Provider: F. Nguyen MD, Gastroenterology

Procedure: Screening colonoscopy  
Indication: Average risk, age 55 at time of procedure.  
Findings: 2 small hyperplastic polyps, sigmoid colon, < 5mm,  
removed by forceps. Remainder of colon normal. Prep: adequate.  
Impression: Hyperplastic polyps, low risk. Repeat in 10 years.  
Signed: F. Nguyen MD 08/14/2023 15:45

[NOTE: This document was auto-included in EHR export. Not directly relevant to current referral.]

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**\*\* RIGHT KNEE MRI NOTE (excerpt - faxed with ortho records 2020) \*\***

[Partial document - remainder of page cut off in original fax]

Patient: Delaney, M. Date: 03/18/2020  
Exam: MRI Right Knee without contrast

Findings: Medial meniscal tear, posterior horn, grade III signal.  
Mild chondromalacia medial compartment. ACL intact. PCL intact.  
Impression: Medial meniscal tear. Recommend orthopedic consult.

[Page cut off - remainder not received]



**\*\*\* FAX TRANSMITTAL SHEET \*\*\***

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TO: Johns Hopkins Neurology Referral FAX: (410) 614-1293  
Intake  
FROM: Shenandoah Valley Regional FAX: (540) 459-8822  
Medical Assoc.  
DATE: 10/03/2024 14:38 PAGES: 14 (including this page)  
RE: Referral - Delaney, Margaret A.  
DOB: 04/17/68

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**MESSAGE:**

Please find enclosed a referral packet for Ms. Margaret Delaney, a 56-year-old female patient of Dr. James Kowalski (Internal Medicine) at Shenandoah Valley Regional Medical Associates.

Ms. Delaney has experienced recurrent episodes of loss of consciousness over the past 8 months (approximately 4-5 episodes). Cardiology workup has been completed and is unremarkable (included in packet). Dr. Kowalski is requesting neurological evaluation.

Enclosed documents include:

- Referral cover sheet
- Patient demographics / insurance
- Referring provider note (10/01/2024)
- Cardiology follow-up note (07/22/2024)
- Emergency department visit summary (09/25/2024)
- Initial PCP visit note (02/14/2024)
- Lab results (02/2024 and 10/2024)
- Imaging: CT head (09/25/2024), Echo (04/10/2024),  
Right knee XR (04/2024) [included in lab/imaging export]
- EEG report (10/02/2024)
- ED discharge summary (09/25/2024)
- Cardiology new patient note (04/08/2024)
- Medication reconciliation
- Additional records (GI, ortho - included in EHR export)

If you have any questions or require additional documentation, please contact our office at (540) 459-8800.

Thank you for accepting this referral.

Sincerely,

S. Hartman RN, Office Coordinator

On behalf of James R. Kowalski MD  
Shenandoah Valley Regional Medical Associates

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